# SCML — SITE COMPLICATION MONITORING LOG

SCML · v0.7

| | | | |
|---|---|---|---|
| Study / Protocol | Site / Code | SCML Record ID / Revision | SCML owner / role |
| Device / System | Procedure Family / Anatomical Field | Record Opened | Last Updated |
| Applicable CIP / Safety Procedure / Current Version | | Study Category Set / Version (if used) | CSST Review Period / Next Review Date |

## A Purpose and use

This log brings together the site's complications, adverse events (AEs), adverse device effects and device deficiencies (DDs) for review across cases. **The Safety and Adverse Effect Reporting Template (SART) provides the individual safety case.** The Intraoperative Checklist and Device Log (ICDL) supplies the procedure and device context.

**Output:** an indexed set of events with a consistent surveillance profile, current source references and linked follow-up. The Complication Signal Surveillance Template (CSST) uses this index to assess patterns; the Principal Investigator (PI) reads its status through the PI Oversight Template (POT).

The SART retains the clinical chronology, causality assessment and reporting details. Record the current classifications and source links here, with enough context to compare cases. Urgent care and safety notification follow the applicable procedure immediately, without waiting for this log or an aggregate review.

## B Recording and Source-Link Guide

| Step | What to record here | Source and boundary |
|---|---|---|
| **Link the event** | C: assign a unique SCML Event ID and enter the SART case ID and specific event reference first. Use participant and surgeon codes. | Keep the same Event ID in C and D. Reference the ICDL and imaging source where relevant; do not copy clinical narratives. |
| **Apply a consistent profile** | D: select the event type, one primary surveillance category and timing group. Copy the current relationship context and DD finding from the SART. | Use Pending where information is unresolved. Use the same category labels across entries; any Other code comes from the study category set. |
| **Review and follow through** | E: cite events or a group and the receiving record. F: check the status and conclusion of an existing action or handoff. | CSST holds rates and the formal signal decision. The SART or linked quality record retains case follow-up, action evidence and effectiveness assessment. |

Open the index while the source case is still being assessed. Update the existing entry when new information changes the same event. A distinct event receives a new Event ID. AE and DD pathways may both apply to one entry; this alone does not create a second clinical event. The Configuration change category refers to an unplanned device configuration change.

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Event and source index

| Study / Site Code | SCML Record ID / Revision | Sheet updated (date) | Sheet No. / Total (C) |
|---|---|---|---|
| | | | |

## C Event and Source Index

Enter one row per distinct event or deficiency. **SART case ID / event reference** links the row to its individual source. Use the same clinical term or study term/code as the source. Record the event date and days from the procedure; enter N/A when the interval does not apply. New information about an existing event updates that entry and its source link.

| SCML Event ID | SART Case ID / Event Reference | Participant Code | Event Term / Study Code | Event Date / Days from Procedure | Device / Component Code | Surgeon Code | ICDL / Procedure Reference | Imaging Reference |
|---|---|---|---|---|---|---|---|---|
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Use the same Event ID in D and mark negative or pending findings explicitly. Keep source details in SART/ICDL; date and initial corrections.

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Structured surveillance profile

| Study / Site Code | SCML Record ID / Revision | Sheet updated (date) | Sheet No. / Total (D) |
|---|---|---|---|
| | | | |

## D Structured Surveillance Profile

**Use the Event ID from C.** Select one event type, primary category and timing group. **Copy relationship contexts, the DD finding and assessment status from the current SART**; multiple relationship contexts may apply. Record the date/initials checked and use Pending while unresolved.

| SCML Event ID | Event Type (from SART) | Primary Surveillance Category | Timing Group | Relationship Context (from SART) | Observed DD? (from SART) | SART Assessment Status / Source Check |
|---|---|---|---|---|---|---|
| | ☐ AE<br>☐ DD without AE<br>☐ AE + DD<br>☐ Pending | ☐ Instability / dislocation<br>☐ Infection / wound<br>☐ Device performance<br>☐ Other code: _____<br>☐ Fracture<br>☐ Neurologic / vascular<br>☐ Configuration change<br>☐ Pending | ☐ Intraoperative<br>☐ Postoperative<br>☐ Other / N/A<br>☐ Pending | ☐ Device<br>☐ Operator<br>☐ Undetermined<br>☐ Procedure<br>☐ Patient | ☐ Yes<br>☐ No<br>☐ Pending | ☐ Pending<br>☐ Preliminary<br>☐ Final<br>Date / initials: _____ |
| | ☐ AE<br>☐ DD without AE<br>☐ AE + DD<br>☐ Pending | ☐ Instability / dislocation<br>☐ Infection / wound<br>☐ Device performance<br>☐ Other code: _____<br>☐ Fracture<br>☐ Neurologic / vascular<br>☐ Configuration change<br>☐ Pending | ☐ Intraoperative<br>☐ Postoperative<br>☐ Other / N/A<br>☐ Pending | ☐ Device<br>☐ Operator<br>☐ Undetermined<br>☐ Procedure<br>☐ Patient | ☐ Yes<br>☐ No<br>☐ Pending | ☐ Pending<br>☐ Preliminary<br>☐ Final<br>Date / initials: _____ |
| | ☐ AE<br>☐ DD without AE<br>☐ AE + DD<br>☐ Pending | ☐ Instability / dislocation<br>☐ Infection / wound<br>☐ Device performance<br>☐ Other code: _____<br>☐ Fracture<br>☐ Neurologic / vascular<br>☐ Configuration change<br>☐ Pending | ☐ Intraoperative<br>☐ Postoperative<br>☐ Other / N/A<br>☐ Pending | ☐ Device<br>☐ Operator<br>☐ Undetermined<br>☐ Procedure<br>☐ Patient | ☐ Yes<br>☐ No<br>☐ Pending | ☐ Pending<br>☐ Preliminary<br>☐ Final<br>Date / initials: _____ |
| | ☐ AE<br>☐ DD without AE<br>☐ AE + DD<br>☐ Pending | ☐ Instability / dislocation<br>☐ Infection / wound<br>☐ Device performance<br>☐ Other code: _____<br>☐ Fracture<br>☐ Neurologic / vascular<br>☐ Configuration change<br>☐ Pending | ☐ Intraoperative<br>☐ Postoperative<br>☐ Other / N/A<br>☐ Pending | ☐ Device<br>☐ Operator<br>☐ Undetermined<br>☐ Procedure<br>☐ Patient | ☐ Yes<br>☐ No<br>☐ Pending | ☐ Pending<br>☐ Preliminary<br>☐ Final<br>Date / initials: _____ |
| | ☐ AE<br>☐ DD without AE<br>☐ AE + DD<br>☐ Pending | ☐ Instability / dislocation<br>☐ Infection / wound<br>☐ Device performance<br>☐ Other code: _____<br>☐ Fracture<br>☐ Neurologic / vascular<br>☐ Configuration change<br>☐ Pending | ☐ Intraoperative<br>☐ Postoperative<br>☐ Other / N/A<br>☐ Pending | ☐ Device<br>☐ Operator<br>☐ Undetermined<br>☐ Procedure<br>☐ Patient | ☐ Yes<br>☐ No<br>☐ Pending | ☐ Pending<br>☐ Preliminary<br>☐ Final<br>Date / initials: _____ |

Relationship contexts reflect the current source assessment. Categories and DD flags are not additional events; follow the CSST counting rule.

# SCML — SITE COMPLICATION MONITORING LOG

Local review and linked follow-up

| Study / Site Code | SCML Record ID / Revision | Sheet updated (date) | Sheet No. / Total (E/F) |
|---|---|---|---|
| | | | |

## E Local Review and Handoff

Log one finding per event or group with a unique Review ID. Select the receiving record and record the decision, linked ID, owner and due date. Use the same Review ID in F. Signal decisions remain in the CSST.

| Review ID / Date | Event IDs / Group Reference | Review Finding / Reason for Handoff | Receiving Record(s) | Decision / Linked Record ID / Owner / Due Date |
|---|---|---|---|---|
| | | | ☐ Safety / SART ☐ CSST<br>☐ SLCT ☐ SRR<br>☐ CAPA ☐ POT | |
| | | | ☐ Safety / SART ☐ CSST<br>☐ SLCT ☐ SRR<br>☐ CAPA ☐ POT | |
| | | | ☐ Safety / SART ☐ CSST<br>☐ SLCT ☐ SRR<br>☐ CAPA ☐ POT | |
| | | | ☐ Safety / SART ☐ CSST<br>☐ SLCT ☐ SRR<br>☐ CAPA ☐ POT | |

## F Linked Action and Handoff Follow-up

Check the existing action in its owning record and cite its outcome and effectiveness conclusion, where applicable. Route a new, overdue or ineffective action through E. Keep a handoff open until the required follow-up is documented.

| Review / Event IDs | Existing Action / Handoff Record ID | Current Status | Owner / Due Date | Source Conclusion / Evidence Reference / Checked Date and Initials |
|---|---|---|---|---|
| | | ☐ Open / on time<br>☐ Overdue<br>☐ Closed<br>☐ Not assessable | | |
| | | ☐ Open / on time<br>☐ Overdue<br>☐ Closed<br>☐ Not assessable | | |
| | | ☐ Open / on time<br>☐ Overdue<br>☐ Closed<br>☐ Not assessable | | |
| | | ☐ Open / on time<br>☐ Overdue<br>☐ Closed<br>☐ Not assessable | | |

## G Current Index Status and Review Conclusion

Select the highest-priority status: Escalated review active → Control action active → Follow-up or assessment pending → Index current. Cite affected Event IDs and records below. Routine CSST review is not escalation; case closure remains in the SART.

| Current Index Status | Select One / Basis | Status Date / Next Review |
|---|---|---|
| **Index current / no pending item** | ☐ Entries are reconciled with their sources; no pending assessment, action or handoff is known. | |
| **Follow-up or assessment pending** | ☐ A source assessment, case follow-up or reconciliation is pending; no higher-priority status applies. | |
| **Control action active** | ☐ A linked corrective or procedural control action remains open; no escalated review status applies. | |
| **Escalated review active** | ☐ An urgent or overdue safety handoff, or a triggered signal review, remains open. | |

Affected Event IDs / Linked Records / Status Basis:

## H Accountable Review and Controlled Record Location

The reviewers confirm that the index was checked against the linked source records and that outstanding handoffs remain identifiable. This review does not replace the investigator's case assessment or any sponsor safety decision. One person may hold more than one site function.

**SCML owner / primary reviewer**

Name / role:

Signature:
Date:

**Clinical / surgical reviewer (if required)**

Name / role:

Signature:
Date:

**Principal Investigator / accountable approver**

Name:

Signature:
Date:

**Next review and controlled record location**

Review date:

Record location:
Current revision: